

NATIONAL PACT FOR QUALITY, EFFICIENCY AND SUSTAINABILITY OF THE HEALTH SYSTEM

At the initiative of the Head of Government, Xavier Espot Zamora,

The Government, the parliamentary groups and the signatory political parties, the management of the Caixa Andorrana de Seguretat Social, the company committee of Caixa Andorrana de Social Security, the management of the Andorran Health Care Service, the schools professionals and the signatory associations and entities in the field of health, as well as the Visura Ciutadana, have decided to strengthen their collaboration to reach an agreement national for the quality, efficiency and sustainability of the health system.

I. Antecedents

1. The basic elements of our health system are attention focused on the person, as the backbone that both lenders and institutions i health administrations must be the focus of all actions and performances sanitary; co-responsibility and the empowerment of people, so that participate in decisions about their health and the management of their own illness and in the financing of treatments and health benefits; equity, because people contribute based on what they enter and use health services in function of what they need; solidarity, understood as society's effort to more vulnerable people have access to the health resources they need, and free choice of professional, because patients can choose which professional will try

It is a system financed according to a social security model, by the CASS and by the Government, and in which the patient participates with a co-payment and takes responsibility also the financing of the system.

The public health system is made up of all health professionals in self-employed who sign an agreement with CASS and all professionals of health in practice on behalf of others contracted by entities with an agreement with the CASS. In addition, you must also count on the network of health centers located in the foreigner with agreements or conventions with Andorra.

To be able to sign an agreement with the CASS to work in the public system is requires accreditation by the Ministry of Health. Health professionals they can also work in the private sector.

The implementation of the preferred route and the obligation to use the clinical history shared by all professionals in the public health system has allowed the evolution towards an integrated and coordinated model between the different levels of care i professionals.

The health indicators we have show that we have one quality and accessible public health care, above, according to criteria of the HAQ index (Healthcare access and quality index),¹ of the average of European countries, in which Andorra ranks ninth in Europe and eleventh in the world.

2. We must be aware, and here lies the importance of this Pact, that ours health system also has shortcomings, which must be identified and resolved in order to improve support and optimize our resources. In this sense, it is necessary to work to strengthen the regulation of health services, improve records and systems of information in order to know the health needs of the population and the state of health of the population, and facilitate access to the health system, both for the user as for professionals, to achieve improvements in equity, attention and long-term sustainability, among others.

The Andorran health system, like most health systems, faces a great challenges The progressive increase in life expectancy has been a great advance; however, the progressive aging of the population generates an increase in diseases chronic conditions and the dependency that entails significant care burdens,

¹ GBD 2019 Healthcare Access and Quality Collaborators. Assessing performance of the Healthcare Access and Quality Index, overall and by select age groups, for 204 countries and territories, 1990-2019: a systematic analysis from the Global Burden of Disease Study 2019; Lancet Glob Health 2022; 10: e1715–43.

health and social. In addition, the new treatments and technologies available mean important advances but they also have economic costs that, in some cases, they can compromise the sustainability of the system.

It is necessary to recognize the important role of home care as a care resource and as a resource to prevent complications and complexity. Therefore, it is necessary to promote the use of resources such as home hospitalization, major ambulatory surgery and palliative care at home, and promote chronic care programs and services.

On the other hand, attention to mental health needs to be reinforced at all stages of the life cycle and fight against the social stigma of these diseases. All these actions are seen reflected in the Comprehensive Mental Health and Addictions Plan (PISMA), approved by the Government in February 2022, which must be deployed and monitored.

3. According to data from the Department of Statistics,² health expenditure stood at 180.5 million euros in 2022, an amount that represented an increase of 0.3% compared to the year 2021. This expenditure represented 5.7% of GDP, while that in 2021 the expenditure was 6.4% of GDP. In terms of spending public health expenditure per capita, in 2021 was €2,262 and in 2022, €2,212.

Public spending on health is growing in a general way, with a trend it is inevitable that healthcare costs will continue to increase progressively, due to the aging of the population, the increase in chronic diseases and the cost of the innovation in health, among other reasons.

4. The health emergency caused by COVID-19 has represented for more than three years a strong threat to people's health and to all systems healthcare, and has committed the human, technical and economic resources in the struggle to cope. It should be noted, however, that despite the undeniable negative impact of the pandemic, the health emergency has also meant opportunities for improvement for the our health system: i) has placed health as a key element for functioning of a country, and consequently health policies and investment in health have acquired a fundamental role; ii) has given a great boost to investment in prevention policies of disease and in public health as a fundamental mechanism to preserve good health

² Statistics of public expenditure on health. Year 2022 (https://sig.govern.ad/SIGDDE.Public/Files/Documents/Notes_premsa_noticies/A040_20230720_A.pdf).

health; iii) has contributed to consolidating the figure of the referring doctor, the use of the shared clinical history of Andorra (HCCA) and to promote the efficient use of resources sanitary; iv) has promoted the development of technological tools applied to the health, such as the Andorra Salut application or the use of telemedicine in consultations medical; v) has allowed the development of research spaces and projects innovators; vi) has demonstrated the resilience, versatility and commitment of staff sanitary; vii) has fostered new mechanisms of collaboration and alliances with others national and international administrations and entities, and viii) has highlighted the reactive and organizational capacity of healthcare professionals, administrations and health institutions with more agile, flexible and adaptable mechanisms than enable a quick response to sudden and critical events.

Our health system has adjusted and responded to the health emergency caused by COVID-19, although learnings and experiences need to be adapted emerged as a result of the pandemic so that we can respond to new emergencies health issues that may arise. In this sense, it is of great relevance to have one action plan to deal with health emergencies that may arise.

5. To solve the existing shortcomings and face the challenges that lie ahead, it is necessary advance in the development of a health system based on four pillars fundamentals:

• The promotion of health and the prevention of disease with the promotion of the habits of healthy lives from childhood and adapted to the different stages of the life cycle and at the different evolutionary or development levels of people, with the aim of delaying or preventing the onset of chronic diseases related to the lifestyle. In addition, it is particularly relevant to enhance the interventions and the design of tools to encourage healthy and dignified aging that allows minimize dependency situations. It is also necessary to promote interventions and tools that improve the quality of life of people in situations of dependence and chronicity.

• Access to quality health care, adapted to health needs of the population, to the scientific evidence and the latest developments available, and with a sufficient number of health professionals with the appropriate skills to provide this assistance.

- The sustainability of the healthcare system, with actions that coordinate and in which the main actors participate, aimed at modifying the systems and the financing mechanisms to make them more efficient and to strengthen the systems of inspection and control to ensure adequate and responsible use of resources sanitary
- The promotion of research and innovation as an instrument to generate knowledge, improve people's health, attract and retain talent and diversify the economy with a sector of great added value.

6. To develop the actions and projects linked to these four pillars, it is necessary have the human and material resources to execute them, as well as tools support as are the new technologies in digital health, a robust information system with access to health data and indicators, laws and regulations adjusted to the reality of the country and a firm commitment from all participants in this Pact to accompany the Government in its deployment.

II. Objectives of the National Pact

The Government, parliamentary groups, political parties, other public entities or parapublics and the associations that sign this National Pact agree that the the following provisions establish the general framework that defines the actions for a the quality, efficiency and sustainability of the healthcare system:

1. Values and foundations

The deployment of the actions of the National Pact is carried out according to the values and following foundations:

- i. The person and the community as the center of the healthcare system. This vision it is incorporated into all actions of the Pact and special emphasis is placed on:
 - a. The empowerment and self-responsibility of the person, promoting the care of one's own health and the relationship with the health system, information and participation in decision-making and the appropriate use of resources sanitary
 - b. The identification of the needs and demands of the citizenry, donor value to the experience of the patient and his environment.

2. Priorities

The priorities set out in this document are accompanied by the tools transversal for the deployment of the actions detailed in section 3.

2.1. Promotion and protection of health and disease prevention

The living conditions and the way of living that people have, individually and collectively, they influence their state of health throughout the entire life cycle. The change in habits related to food, the practice of physical activity or abandoning the consumption of addictive substances allows morbidity to be reduced and mortality from chronic diseases associated with these risk factors, as they are cardiovascular diseases, diabetes or some types of cancer. In this sense, special emphasis must be placed on the promotion of healthy lifestyles from childhood, so that people can incorporate these habits from the earlier stages and maintain them throughout the life cycle. It is also necessary to promote all the actions aimed at healthy ageing, with the aim of maintaining over time the health and autonomy of people.

It is also necessary to continue to develop and strengthen the actions that allow prevention the onset of diseases or to diagnose them in early stages when there are more margin of intervention and health outcomes are much more favorable. At this point vaccination programs and campaigns as well as programs should be highlighted of breast and colon cancer screening and all actions aimed at preventing and early detection of mental health problems, combating suicide and also of fighting against the stigma that is related to mental health.

Health promotion and disease prevention actions must have a vision transversal, with an approach based on the "one health" concept, in which the actors involved in human, animal and plant health are involved and coordinated and environmental with the purpose of promoting well-being and neutralizing threats for health and ecosystems; and, at the same time, attend to the collective need of drinking water, energy and air, healthy and nutritious food, take action against climate change and contribute to sustainable development.³

³ www.who.int

The main lines that must be developed or consolidated are, among others:

- a) Assess the state of the situation through health surveys.
- b) Promote healthy lifestyles: promote healthy eating and physical activity practice.
- c) Promote active and healthy aging.
- d) Prevent and support the abandonment of toxic consumption through the Plan national against drug addictions (PNCD).
- e) Encourage mental health, prevention and early detection of health problems mental health, the fight against stigma, and the development of a national strategy of prevention and fight against suicide.
- f) Strengthen epidemiological surveillance, as well as disease control transmissible, outbreaks of any cause and emerging diseases.
- g) Carry out vaccination programs and campaigns.
- h) Launch early detection programs.
- i) Ensure food safety and water safety for human consumption.
- j) Ensure air quality.
- k) Carry out sexual and reproductive health programs and projects.
- l) Implement a plan for preparation and response to health emergencies public.
- m) Promote and make the donation of tissues and organs effective.
- n) Promote the rational use of medicines and especially antibiotics.

2.2. Access to quality healthcare

WHO⁴ defines quality in health care as the degree to which services increase the likelihood of desired health outcomes. With this one objective, quality health services must be effective, safe, focused on the person, timely – so that waiting times and delays are reduced assistance – equitable, integrated and efficient.

⁴ https://www.who.int/es/health-topics/quality-of-care#tab=tab_1

To be able to continue to have quality healthcare and improve the health outcomes, it is necessary to have health professionals in sufficient numbers and with the appropriate skills and healthcare adapted to needs of the population, to the scientific evidence and to the latest developments available.

Human resources in health

It is of particular relevance to carry out a planning of health professionals necessary in the medium and long term to allow actions to be taken to prevent future ones distortions, through actions for the recruitment and retention of talent.

As measures aimed also at the recruitment and retention of talent are proposed **improvements** in the remuneration system for health professionals, as well as the obligation of continuing education, which will also affect the quality of health care.

The main lines that must be developed or consolidated are, among others:

- a) Plan the human resources needed in the medium and long term, with the identification of current and future needs, the establishment of actions that it is necessary to carry out, its development and its evaluation.
- b) Establish continuing education programs in coordination with schools professionals and with reference centers, addressed to all actors in the system health: health professionals, entities and agents of civil society and third parties sector involved in health projects.
- c) Promote work and communication between health professionals both in the country and of reference centers. In this sense, the system must establish networks of agile interprofessional collaboration.

Health care

The portfolio of services and health products establishes the set of acts, the products and the benefits offered to the population in terms of health having in takes into account quality, safety and efficiency criteria. This portfolio should be reviewed and update continuously with the aim of adapting it to the needs of the population, to available advances and technologies and to scientific evidence.

To ensure continuity in quality care, review and improvement are proposed continuation of the spaces, technology and procedures of the different levels assistance

To respond to the serious problem posed by health problems mental for the people who suffer from them, their families and society in general, it is proposed to develop what is planned in the PISMA, as well as improve and adapt available resources to existing needs.

In relation to oncological care, it is proposed to improve the oncological care network with the aim of offering comprehensive support to the patient throughout the process of diagnosis, treatment and follow-up.

It is also established as a priority to develop actions aimed at a better one care for people with disabilities and functional diversity or with illnesses chronic conditions, especially those that are more prevalent, more serious or that generate more loss of autonomy

And when situations of dependency occur, care models must be developed integrated social and healthcare, which focus on the person and not on the areas of action

To improve efficiency, it is planned to strengthen the role of health agents, who can have a relevant role in health promotion, disease prevention, the diagnosis, treatment, rehabilitation and support and recovery of the illness

The main lines that must be developed or consolidated are, among others:

a) Continuously review and update the portfolio of services and include them new features

b) Primary and community care:

- i. Strengthen the figure of the referring doctor.
- ii. Emphasize and enhance the key role of health agents involved in primary and community care, and promote their coordinated work and integrated with the rest of the care levels.
- iii. Improve the network of primary care centers.

- iv. Carry out actions to have knowledge and care of people who they live in solitude, in coordination with the Ministry of Social Affairs.

c) Hospital care:

- i. Hospital technology.
- ii. Make an extension of the HNSM on the adjacent plot.
- iii. Reduce hospital stays: encourage home hospitalization i increase ambulatory major surgery, among others.

d) Urgent and continuous care:

- i. Consolidate the separation of the SUM and the Emergency Service.
- ii. Streamline the use of the Emergency Service.
- iii. Analyze and monitor medical transport.

e) Palliative care:

- i. Promote palliative care at home, promoting networking between primary and hospital care.

f) Comprehensive attention to mental health:

- i. Deploy, implement and evaluate the Comprehensive Mental Health Plan and addictions
- ii. Consolidate outpatient psychology with public funding.
- iii. Develop the housing model in mental health.
- iv. Introduce a network support system for families of people with mental health problems.
- v. Promote psychological care for people and their families with serious, chronic or disabling illnesses.

g) Comprehensive care for cancer patients:

- i. Improve the oncology care network to offer comprehensive support of the patient and the family throughout the process of diagnosis, treatment and tracking

h) Comprehensive attention to chronicity and dependency:

- i. Define a model of integrated social and health care, in coordination with the Ministry of Social Affairs and with the participation of civil society.
- ii. Establish care routes and protocols for more chronic diseases prevalence

i) Develop comprehensive attention to sexual and reproductive health.

2.3. Sustainability and equity of the healthcare system

As has been explained, in recent years healthcare expenditure has been increasing progressively, due mainly to the aging of the population, to the increase in the prevalence of chronic diseases and also to the same evolution of medicine and health technologies.

The introduction, in 2019, of the preferred route was carried out with the aim of improve the quality of health care but also to make ours more sustainable healthcare system So, almost five years have passed since it was put on operation, it is necessary to analyze its operation with all the actors and establish the changes and adjustments that contribute to improving health care and sustainability of the system.

Considering the key role in the sustainability of the health system of the mechanisms of funding, it is necessary to review them and explore additional mechanisms based on them results and procedures of strategic interest to the system.

The sustainability of the healthcare system depends on the commitment of the professionals, the managers and citizens. All actors must make rational use of resources, since are limited, with the aim of ensuring the viability of a public health system, equitable and quality.

The main lines that must be developed or consolidated are, among others:

- a) Analyze the operation of the preferred route and make proposals for improvement.
- b) Evaluate the use of new technologies as tools to improve access to care healthcare and the sustainability of the system (e.g. online visits), taking into account the digital divide.
- c) Establish new financing mechanisms giving value to the processes complex and attention to complexity, and review the participation system of the user in financing.
- d) Strengthen the inspection and control system.
- e) Consolidate monitoring and evaluation, as an essential element for the dam of decisions.

2.4. Research and innovation

An essential aspect for improving the health of the population is to promote research and innovation in health with the aim of finding new mechanisms to improve health of the population, prevent diseases, develop faster diagnostic tools, more sensitive and specific, as well as more effective treatments, enhancing the use of personalized medicine and innovation in health technologies.

In addition to the benefits for people's health, the development of the sector of research and innovation can generate significant economic benefits, educational and social.

The main lines that must be developed or consolidated are:

- a) Promote an ecosystem conducive to research and innovation by promoting networks of collaboration with reference centers.**
- b) Encourage the participation of Andorran professionals in research projects for also favor the recruitment and retention of talent.**
- c) Promote the establishment of companies and entities in the field of research and innovation with the aim of creating a research and development environment.**

3. Tools for the deployment of actions

To carry out the deployment of the actions provided for in the National Pact, it is necessary have the necessary human, material and financial resources, as well as the supporting tools detailed below:

3.1. Technological tools: digital health

The main objective of the digital transformation in the field of health is to put the citizen at the center of the healthcare system, favoring their autonomy in relation to the management of your health information and with the monitoring of your processes assistance In addition, it is necessary to promote the use of standardized terminology and the interoperability between the different tools. At the same time, it is essential to have one internal and external health data management system: Andorra Data Hub.

a) Shared clinical history

The shared clinical history is a solid, safe and reliable technological support, essential to have an integrated and coordinated health care model between the different levels and actors. In this sense, it is necessary to keep moving forward to integrate all the health professionals, to incorporate the social determinants, to facilitate them the use and to grant new functionalities to this tool.

b) Prescription and electronic prescription

Prescription through the shared clinical history and the integration of pharmacies and the CASS in the electronic prescription system are key measures to improve the safety in the use of medicines and avoiding unnecessary consumption and misuse of medicines. This strategic project will also have an impact on the analysis and the monitoring the consumption of medicines, as well as in the sustainability of the system sanitary.

c) New features of the Andorra Salut app

The Andorra Salut app has proven to be a very useful tool for the population, especially during the health emergency caused by the COVID-19, and so he puts it of course the number of downloads of this tool (until April 2023 25,000 people have generated nearly 40,000 downloads⁵).

Beyond the functionalities of disease, vaccination, test tests and certificates related to COVID-19, the Andorra Salut app incorporates other functionalities, can ara hospital discharge and emergency medical reports, agenda, prescription email and the request for an appointment at SAAS.

However, progress must be made to add new functionality and integrate with it services of other health actors in the country, which allow Salut Andorra to become the app of reference in the field of health, improve the efficiency of the health system and place the user at the center of the system and as the protagonist of their health.

⁵ Department of Computer Science. SAAS.

d) New applications and devices (eg wearables, virtual reality, AI)

In the same way as the Andorra Salut app, advances in digital health offer devices, applications and technologies such as artificial intelligence (AI) that are needed to explore, regulate and use in health promotion and disease prevention but also integrate them into diagnostic, monitoring and care processes.

3.2. Information system

Have a robust, reliable, practical and up-to-date information system formed by validated health indicators make it possible to know the health needs of the population, the state of the healthcare system and plan, prioritize and adjust the actions that need to be implemented. At the same time, it is necessary to establish secure mechanisms for the management of data and its interoperability

3.3. Rules and agreements

The projects and actions provided for in this National Pact can only be carried out in term if you count the legal texts that accompany the development of actions and that make it possible to offer the necessary guarantees to citizens, companies, health providers and health managers. In the same way, you need to count with a network of cooperation with the countries and entities of our environment and experts in the matters subject to the Pact.

- a) Reinforcement of cross-border healthcare collaboration.**
- b) Agreements with organizations and entities of other states.**
- c) Medicine Law.**
- d) Law on organization of the pharmaceutical sector.**
- e) SAAS Act.**
- f) Law on the implementation of the Framework Convention against tobacco.**
- g) Health Oblivion Law.**

III. Methodology

With the firm will to monitor the status of the actions set in this National Pact, it is agreed to establish the following work methodology:

- Regular working meetings between the parties designated to deal with specific issues and follow up.**
- Submission of documentation with the state of the actions.**

IV. commitment

1. The Government, the parliamentary groups and the signatory political parties, the management of the Caixa Andorrana de Seguretat Social, the company committee of Caixa Andorrana de Social Security, the management of the Andorran Health Care Service, the schools professionals and the signatory associations and entities in the field of health, as well as the Visura Ciutadana, share the will to work and cooperate for excellence and the efficiency of the health system, so that it is enduring over time and that give the best and most efficient response to the needs of our citizens and emergencies and other health crises that may arise, and in this sense it is undertake to strengthen their unity in this area and to work together in benefit of Andorran society and the country.

2. Consequently, by signing this National Quality Pact, the efficiency and sustainability of the health system, the Government, the parliamentary groups, political parties, other public or parapublic entities and associations that sign this National Pact publicly ratify their commitment.

The signatories of this agreement are detailed below, notwithstanding that they may be add, after the date of signature, other signatories invited to form part of this Agreement.

Andorra la Vella, July 8, 2024